

# LENS Prompt Library

*Every demo prompt in the course, ready to copy, paste, and adapt. Each one is built with LENS: Lead with the persona, Establish the context, Name the task, Set the constraints.*

## S01 C02 Prompt: translate Six Sigma jargon

You are a Lean Six Sigma expert who is great at explaining things simply. I am a brand new white belt with no Six Sigma background. Below is an email I just received from my green belt. Please translate it for me into plain English. Explain every technical term you encounter, in parentheses, as you go. Then at the end, give me a one-paragraph summary of what the email is telling me. Here is the email:

## S01 C03 Prompt pack: jargon translator

You are a Lean Six Sigma expert who is great at explaining things simply. I am a brand new white belt with no Six Sigma background. Below is an email I just received from my green belt. Please translate it into plain English. Explain every technical term in parentheses as you go, including every acronym. Then give me a one-paragraph summary of what the email is actually telling me. Here is the email:

## S08 C44 Weak vs strong: the two prompts

### Weak prompt

Help me with my Six Sigma project.

### Strong prompt

Act as a senior Lean Six Sigma Green Belt who explains things simply. I was just added to my very first Six Sigma project at a parcel delivery company that is trying to reduce late deliveries, and I want to understand the whole DMAIC roadmap before I dive in. In plain English, give me a quick cheat sheet of the five DMAIC phases, with one short line for each on what actually happens. No academic jargon, and keep it under 120 words.

## S09 C49 Decode a charter: prompt

You are a Lean Six Sigma expert and a friendly mentor. I am Priya, a brand new white belt who works in operations at a regional health system, and I have four years of frontline experience in the emergency department. Marcus, my green belt, has added me to a project to reduce ED door-to-doctor time across our three campuses, and below is the project charter he sent me. Please do four things. First, summarize the charter in three short paragraphs in plain English, defining any technical term in parentheses the first time it appears. Second, given my background, tell me what my specific role on this project is likely to be. Third, draft three

smart questions I could ask Marcus at the kickoff that show I read the charter carefully. Fourth, tell me anything in the charter you are less confident about, so I know what to double check. Here is the charter.

### **S10 C55 Organize a process description: prompt**

You are an experienced process mapper. I am Priya, a white belt on a Six Sigma project to reduce ED door-to-doctor time, and I have four years of frontline ED experience. Below is my rambling description of how patients flow through our emergency department. Please do three things. First, turn it into a clean numbered list of steps in the order they actually happen, including the edge cases I mentioned. Second, ask me five specific follow-up questions that would tighten the description, especially exceptions or steps I might have skipped. Third, suggest a high-level flowchart structure with parallel paths for the edge cases. Here is my description.

### **S11 C61 Compile VOC: prompt**

You are a Lean Six Sigma expert. I am Priya, a white belt on a project to reduce ED door-to-doctor time at our regional health system. Below are three sources of patient voice: a satisfaction survey export, a complaint log, and a patient relations email archive. Please compile them into one tidy reference document with this structure. Section one, survey responses, in their original order, each numbered, the original wording preserved in quotation marks, with the satisfaction rating in parentheses after each quote. Section two, complaint log entries, in original order, each numbered, wording in quotation marks, with the complaint category as recorded in the log. Section three, patient relations email replies, in original order, each numbered, with the key patient concern in quotation marks. Do not cluster. Do not theme. Do not interpret. Compile only. Each patient voice should appear once, in the section that matches its source. Here are the three sources.

### **S12 C67 Build a Process Map Card**

You are a Lean Six Sigma facilitator helping me build a process map. I am Priya, a white belt on a project to reduce ED door-to-doctor time. Here is roughly how a patient moves through our emergency department. They arrive and check in at the front, go to triage for vitals and an acuity level, then usually to registration, then they wait for a bed, then a nurse assessment, then the physician sees them, then tests and results. Sick patients skip registration and register at the bedside, and low acuity patients can take a separate fast track path. Please give me the steps in order, identify who owns each step, flag any decision points, call out anywhere you see a likely delay or a gap, and then draw the whole thing as a flowchart.

### **S13 C73 Build and Read a Histogram Card**

You are a statistics tutor. I am Priya, a white belt on a project to reduce ED door-to-doctor time. Here are the door-to-doctor times in minutes for two hundred patients. Please create a histogram so I can see the distribution. Choose sensible buckets for me. Then describe the shape of the distribution in plain English, and tell me if you see anything I should investigate, like skew or more than one peak.

### **S14 C79 Build a Run Chart Card**

You are a statistics tutor. I am Priya, a white belt on a project to reduce ED door-to-doctor time. Here are twenty weeks of average door-to-doctor time in minutes, in order from week one to week twenty, for our South campus. Please create a run chart. Then tell me in plain language whether you see any real signals, like a sustained trend or a sudden sustained shift, or whether this looks like normal week to week noise.

## **S15 C85 Fishbone and Five Whys cards**

### **Fishbone brainstorm**

You are a Lean Six Sigma facilitator. I am a white belt running a root-cause brainstorm with a fishbone diagram. The effect we are trying to explain is that patients wait too long from arrival to first being seen by a physician in our emergency department. Please brainstorm possible causes, organized under the six categories, Method, Machine, Material, Manpower, Measurement, and Environment. Give me several specific, plausible causes under each, and do not hold back, I want a wide net.

### **Draw the diagram**

Now draw this as a fishbone diagram. Put the effect on the right, the six categories as the bones, and the causes you listed along each bone, so I can copy it into my project file. Ask the chat tool to draw it, do not send it to an AI image generator, because image generators paint pictures and garble the labels.

### **Five Whys**

Let us do a Five Whys on the cause that admitted patients board in the ED. Ask me why five times to push toward a root cause.

## **S16 C91 Build a Pareto card**

You are a Lean Six Sigma expert. I am a white belt on a project to reduce ED door-to-doctor time. Here are three hundred short free-text notes on what most delayed each patient. Please group them into a small set of sensible categories, count how many fall into each, and create a Pareto chart with the categories sorted from most frequent to least, plus a cumulative percentage line. Then tell me which categories make up roughly the first eighty percent of the volume.

## **S17 C97 Divergent and convergent cards**

### **Divergent (brainstorm a range)**

You are a Lean Six Sigma expert. I am a white belt. Our root cause is this. When a patient is admitted to the hospital from the emergency department, no one is clearly in charge of arranging their room upstairs, so they get stuck waiting in an emergency department bed, which fills up the department and makes new patients wait longer to be seen. Brainstorm a range of possible solutions, from very simple and low tech to more complex, and for each note roughly how much effort it would take.

### **Convergent (skeptical stress test)**

Act as a skeptical ED operations manager. Here is my proposed solution, naming a charge nurse each shift to own the bed request. Tell me every way this could fail or backfire in practice.

### **S18 C103 Draft an SOP card**

You are helping me write a standard operating procedure. I am Priya, a white belt on a project to reduce ED door-to-doctor time. Here is roughly how our new process works. The moment an emergency department doctor decides to admit a patient to the hospital, the charge nurse on duty is in charge of getting that patient a room upstairs. They note the time the decision was made, ask the unit upstairs for a room right away, and check back every thirty minutes until a room is assigned. If no room is free within two hours, they raise it to the house supervisor, the senior nurse who oversees the whole hospital that shift. And when one shift ends and the next begins, the charge nurse going off duty hands any room requests that are still open to the charge nurse coming on. Please turn this into a clear, professional SOP with a title, a purpose, a scope, the roles involved, and a numbered step-by-step procedure.

### **S19 C109 Build a control chart**

You are a patient statistics tutor. I am Priya, a white belt monitoring our ED improvement. Here are thirty days of daily average door-to-doctor time in minutes, in date order. Please create a control chart. Calculate the center line and the upper and lower control limits using three standard deviations. Plot the points, then tell me whether the process is in statistical control, flagging any points outside the limits or any other out-of-control patterns, like a long run of points on one side of the center line.

### **S20 C115 Draft a response plan**

You are a Lean Six Sigma expert. I am Priya, a white belt. Help me write a simple response plan for our control chart. Every morning we look at a control chart of the average time it takes a patient to be seen by a doctor in our emergency department. The lead charge nurse is the one who checks it. We treat it as a signal if any point lands outside the limits, or if eight points in a row sit on one side of the center line. When that happens, I want the lead charge nurse to look into the cause, starting by checking whether the procedure we wrote for getting admitted patients a room upstairs is actually being followed, then tell the ED operations manager, write down what they found and what they did about it, and confirm the chart settles back to normal. Please format this the way it would appear in a real control plan: a short purpose line, then a response plan TABLE with columns for Signal, First Check, Who to Notify, Corrective Action, and What to Record, followed by a simple response log table the team can fill in when a signal occurs. Keep it to one page.